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280

healthcare workers identified and offered support

9/9

sentinel event RCAs with second victim follow-up

136

Wellness Warriors trained across 57 units

8/11

international consensus indicators achieved

 Background

- Patient safety incidents can turn involved healthcare workers (HCWs) into **second victims**: psychological distress, burnout, and loss of professional confidence.
- Most support programmes depend on **self-referral**, so the affected population remains unknown.
- International consensus now uses *support provided / second victims identified* as a core indicator,¹ yet reports from Southeast Asia are scarce.

 Objective & Methods

To describe how Wellness Warriors are integrated with second victim management and root cause analysis (RCA), and to map programme performance against 11 international consensus indicators.

Design

Descriptive implementation study, document review

Setting

RSCM, Indonesia's national referral hospital (~5,300 staff, 57 units)

Period

January to August 2026

Framework

Scott's three-tiered model;² 25-item Indonesian SVEST;³ 11 consensus indicators¹

 Wellness Warriors Support Pathway

TIER 1 · UNIT-LEVEL IDENTIFICATION

Quality Team & IKL-K3RS

1

Sentinel / adverse event occurs and an incident report is filed.

2

The **Quality Team** (Risk Management, Quality and Cost Control) conducts a **non-punitive RCA**.

3

IKL-K3RS joins the RCA and identifies staff involved who may be second victims.

4

IKL-K3RS sends **SVEST** to the unit and to the Wellness Warriors.
IKL-K3RS ↔ Wellness Warriors

TIER 2 · PEER SUPPORT

Wellness Warriors

5

Wellness Warriors give **Psychological First Aid (PFA)** and distribute SVEST to affected staff.

6

Staff complete SVEST and return it via Wellness Warriors to IKL-K3RS.

TIER 3 · PROFESSIONAL SUPPORT

Occupational Medicine

7

Occupational Medicine reviews SVEST scores across 7 domains.

8a · All domains below cut-off

Report and close follow-up.

8b · Any domain ≥ cut-off

Occupational Medicine interview (fitness to work), then referral to a psychiatrist.

9

Reporting: IKL-K3RS → Quality Team → Board of Directors.

IKL-K3RS: Environmental Health and Hospital Occupational Safety and Health Installation · PFA: Psychological First Aid · SVEST: Second Victim Experience and Support Tool



SVEST: 25 items, 7 domains

Likert 0 to 4. Positively worded support items are reverse-scored, so a higher score always means a worse condition. Cut-off = 75% of each domain maximum.

Psychological distress	≥12/16
Physical distress	≥12/16
Colleague support	≥12/16
Supervisor support	≥12/16
Institutional support	≥9/12
Non-work-related support	≥6/8
Professional self-efficacy	≥12/16

 Results

280

identified and offered support

202

staff in 6 of 9 completed cases

32

SVEST returned (15.8%)*

0

at or above any cut-off

- Median 32 HCWs per event (range 18 to 52)
- Most affected: **trainees and nurses**
- 3 of 9 cases still in progress
- Mean time from incident to first contact: **[__] days**

*Response rate = 32 / 202 × 100% = 15.8%; data collection ongoing.

 Consensus Indicators¹

8/11

achieved

4 newly introduced (NEW)

- Awareness
- Process / structure
- Impact
- Not yet measured

AWARENESS

✓ Support / activation requests

9/9

✓ Support / SV identified **NEW**

280/280

✓ HCWs aware of programme

100%

PROCESS / STRUCTURE

✓ Time from incident to first contact **NEW**

[__] d

✓ Approved institutional policy

SOP

✓ Trained / total peer supporters

136/136

✓ HCWs trained on SV topic **NEW**

57 units

IMPACT

✓ Perceived benefit after peer support: support from supervisors, colleagues, organisation

○ Distress before and after support

○ Qualitative feedback from SV

○ Lost workdays, SV vs total **NEW**

 Discussion

From self-referral to proactive case-finding. Linking second victim identification to every sentinel event RCA produced a known denominator of 280 HCWs across 9 events. This puts into practice the observation that institutions can screen at-risk professionals immediately after an event,⁴ and makes the consensus indicator *support provided / second victims identified* measurable.¹

Peer support first. In the SVEST validation study, the most desired form of support was a respected peer to discuss what happened.³ Wellness Warriors (136 in 57 units) put this into practice as Tier 2, in line with the European recommendation for peer support programmes.⁵

Patient safety and worker well-being in one pathway. Embedding support in a non-punitive RCA connects incident learning with staff care and supports a just culture.⁵

Early SVEST data. Returns are still low (15.8%) because collection is ongoing, and no respondent has reached a domain cut-off so far. These early results should be interpreted with caution.

Next step: impact indicators. Second victim distress has been linked to turnover intention and absenteeism, mediated by organisational support.⁶ Measuring distress before and after support, qualitative feedback, and lost workdays is the next priority.

Limitations. Single-centre document review; the Indonesian SVEST validation is unpublished institutional data.

Conclusion

Integrating Wellness Warriors, second victim management, and system-based RCA establishes a non-punitive organisational framework. It can be implemented in hospitals with existing incident review structures and is a foundation for sustainable healthcare worker well-being.

References

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Indonesian SVEST validation: unpublished institutional data.